

Clinical Study Summary Report (CSSR)

Document Status: Final | Version: 1.0 | Date: 01-APR-2026

1. Administrative & Study Identification

Full Study Title:	A Phase III, Randomized, Open-Label Study Evaluating the Efficacy and Safety of Divarasib and Pembrolizumab Versus Pembrolizumab and Pemetrexed and Carboplatin or Cisplatin in Patients With Previously Untreated, KRAS G12C-Mutated, Advanced or Metastatic Non-Squamous Non-Small Cell Lung Cancer
Short Title:	A Study Evaluating the Efficacy and Safety of Divarasib and Pembrolizumab Versus Pembrolizumab and Pemetrexed and Carboplatin or Cisplatin in Participants With Previously Untreated, KRAS G12C-Mutated, Advanced or Metastatic Non-Squamous Non-Small Cell Lung Cancer
Protocol Number:	1096784258430081179
NCT Number:	NCT06793215
Sponsor Name:	Roche
Investigational Product:	cisplatin
Phase:	PHASE3
Medical Monitor:	[Not Provided in Posting]

2. Study Synopsis & Rationale

2.1 Study Objective

Primary Objective:	1. Progression-Free Survival (PFS): PFS is defined as the time from randomization to the first occurrence of disease progression, as determined by blinded independent central review (BICR) according to RECIST v1.1, or death from any cause (whichever occurs first) 2. Overall Survival (OS): OS is defined as the time from randomization to death from any cause
Secondary Obj.:	1. Objective Response 2. Change from Baseline on the European Organisation for Research and Treatment of Cancer Quality-of-Life Questionnaire-Supplemental Lung Cancer Module (EORTC QLQ-LC13) Cough Scale 3. Change from Baseline on the EORTC Quality of Life Questionnaire (QLQ-C30) Dyspnea Item and Physical Functioning Scale

2.2 Background & Rationale

The purpose of this study is to evaluate the efficacy and safety of divarasib and pembrolizumab compared with pembrolizumab and pemetrexed and carboplatin or cisplatin, for the first-line treatment of adult participants with KRAS G12C-mutated, advanced or metastatic non squamous non-small cell lung cancer (NSCLC).

3. Study Design & Methodology

Design Framework:	Type: INTERVENTIONAL, Phase: PHASE3, Status: RECRUITING
Target N:	600

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4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria

- * Eastern Cooperative Oncology Group (ECOG) performance status of 0 or 1
- * Histologically or cytologically confirmed diagnosis of advanced or metastatic non squamous NSCLC that is not eligible for curative surgery and/or definitive chemoradiotherapy
- * Measurable disease, as defined by RECIST v1.1
- * No prior systemic treatment for advanced or metastatic NSCLC
- * Documentation of the presence of a KRAS G12C mutation
- * Documentation of known PD-L1 expression status in tumor tissue
- * Availability of a representative tumor specimen
- * Adequate end-organ function
- * Eligible to receive a platinum-based chemotherapy regimen

Exclusion Criteria Related to NSCLC:

- * Known concomitant second oncogenic driver with available targeted treatment
- * Symptomatic, untreated, or actively progressing central nervous system (CNS) metastases
- * Spinal cord compression not definitively treated with surgery and/or radiation or previously diagnosed and treated spinal cord compression without evidence that disease has been clinically stable for ≥ 2 weeks prior to randomization
- * History of leptomeningeal disease
- * Uncontrolled tumor-related pain
- * Uncontrolled pleural effusion, pericardial effusion, or ascites requiring recurrent drainage procedures (once a month or more frequently)

Exclusion Criteria Related to Current or Prior Treatments:

- * Any anti-cancer systemic therapy, including hormonal therapy, within 21 days prior to randomization, or is expected to require any other form of antineoplastic therapy while in the study
- * Radiation therapy including palliative RT to bone metastases within 2 weeks prior to randomization and RT to the lung >30 Gy within 6 months prior to randomization
- * Prior treatment with KRAS G12C inhibitors or pan-KRAS/RAS inhibitors
- * Treatment with systemic immunosuppressive or immunostimulatory medications, including CD137 agonists and immune checkpoint inhibitors
- * Current treatment with medications that are well known to prolong the QT interval
- * Treatment with therapeutic oral or IV antibiotics within 2 weeks prior to randomization
- * Prior allogeneic stem cell or solid organ transplantation

Exclusion Criteria Related to General Health:

- * History of malignancy other than NSCLC within 5 years prior to screening, with the exception of malignancies with a

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negligible risk of metastasis or death (e.g., 5-year overall survival \[OS\] rate \>90%), such as adequately treated carcinoma in situ of the cervix, non melanoma skin carcinoma, localized prostate cancer, ductal breast carcinoma in situ, or Stage I uterine cancer

* Individuals with chronic diarrhea, short bowel syndrome or significant upper gastrointestinal surgery including gastric resection, a history of inflammatory bowel disease (e.g., Crohn's disease or ulcerative colitis) or any active bowel inflammation (including diverticulitis), malabsorption syndrome, conditions that would interfere with enteral absorption

* History of idiopathic pulmonary fibrosis, organizing pneumonia (e.g., bronchiolitis obliterans), drug-induced pneumonitis, or idiopathic pneumonitis, or evidence of active pneumonitis on the screening chest computed tomography scan

* Significant cardiovascular disease within 3 months prior to screening

4.2 Exclusion Criteria

Not Provided

11. Study Identifiers & Governance

Primary ID:	1096784258430081179
Registry Number:	NCT06793215
Sponsor:	Roche
Study Title:	A Study Evaluating the Efficacy and Safety of Divarasib and Pembrolizumab Versus Pembrolizumab and Pemetrexed and Carboplatin or Cisplatin in Participants With Previously Untreated, KRAS G12C-Mutated, Advanced or Metastatic Non-Squamous Non-Small Cell Lung Cancer

15. Sign-off & Approval

Principal Investigator: _____	Date: _____
Clinical Operations Lead: _____	Date: _____
Lead Statistician: _____	Date: _____